CUSTOMER EVIDENCE & VALIDATION

SEIC 2026 – Cohort 2 | Session: Problem Validation & Market Research – 23 October 2026 | Instructor: Sana Shaharyar, Director ClickChain AI

Startup: Happy Health – Home Medical Care, Karachi – Idea Stage	Founder: Theo – Solo – DHA/Clifton

D. CUSTOMER EVIDENCE LOG

Record what people actually said or did. Separate direct evidence from your interpretation. Use participant IDs (P01, P02…) rather than sensitive personal details.

Participant ID	Problem / observed behavior	Frequency / impact	Current workaround
P01 – DHA Phase 6 – Son 38 – Father 72 bedridden diabetes 32.9%	Last time father needed daily care, called 3 numbers from WhatsApp group DHA, got maid who claimed RN but no PMDC license. No vitals log, no family sign proof. Said: 'I paid Rs.95k to maid, no BP log, had to call 3 times, wasted whole day.'	Daily – 10 hrs/week coordinating + 1 hospital trip/week Rs.5k + Rs.95k/mo maid. Quantified cost Rs.35,750/week.	Unqualified maid Rs.95k-120k/mo (Lajpal benchmark), hospital queue for labs, mother caregiver overloaded, no dashboard.
P02 – Clifton – Daughter 35 – Mother 68 HTN 30.4%	Mother needs BP + sugar daily, oladoc Rs.500 per visit, no continuity. Said: 'Every time nurse comes, different person, no record of yesterday's BP.'	Daily vitals – 7x/week, Rs.500×7=Rs.3,500/week oladoc + time. Impact: No trend tracking, risk wrong medicine.	Oladoc per visit Rs.200-5k, Rozee listing no verification, WhatsApp group search, neighbor help.
P03 – Overseas – Dubai son 40 – Father DHA 70	In Dubai, father in DHA fell, needed ECG within 24h. Said: 'I was in Dubai, had to book flight Rs.80k + 2 weeks leave Rs.100k, no one in Karachi to manage.' No dashboard, no WhatsApp updates.	Urgent crisis – occurs 1-2×/year per overseas family, but when occurs cost Rs.180k + job risk. 52.6% private spend, 89% OOP.	Video call mother, ask neighbor, pay hospital home visit USD package AKU, no verified pool. Wants Family Health Dashboard + 48h reminders + yearly planner.
P04 – PECHS – Post-surgery discharge AKU – Daughter 32	Father discharged from AKU post-surgery, needed dressing + medicine + bed within 24h. Said: 'AKU discharged at 4pm, we searched till 10pm, no nurse with bed.'	Post-discharge – occurs once per surgery, but urgent <2h SLA needed. Risk readmission Rs.150k.	AKU home care referral none, Lajpal no equipment, Chughtai HB Rs.550 free sampling available but no integration, equipment rental market Rs.1k/day BP no deposit SOP.
P05 – Gulshan – Mother 60 caregiver – Equipment risk	Mother caregiver for father, rented BP monitor, damaged, shop demanded Rs.15k. Said: 'No deposit slip, no SOP, we lost money.' Equipment loss risk 2% noted.	Equipment need – 2-3×/month per family, Rs.500/day BP, Rs.15k/mo bed, Rs.1,500/day ECG. Deposit Rs.7,500 avg needed, liability Rs.75k for 10 items.	Market rental without deposit SOP, no check-in/out log, no family sign proof. Wants deposit SOP + GPS + family sign.

What patterns or recurring themes did you notice?	1. Verification is core wedge: 5/5 participants mentioned 'no PMDC license check' + 'no hospital reference + police' – Rozee/OLX listing model fails trust. 2. On-time <2h SLA with zone hubs DHA/Clifton + Gulshan/PECHS + GPS is missing – Human Healthcare 2022 model (license + call hospitals + background + senior supervises first visit + zone hubs). 3. Documentation gap: AKU CarePartner documents all services, but home care has no vitals log + medicine log + family sign proof + WhatsApp updates + 48h reminders + yearly planner. 4. Overseas needs dashboard: Dubai/UK/US sons want Family Health Dashboard + peace of mind, willing to pay Rs.60k-100k/mo via bank transfer. 5. Cost quantified: Rs.35,750/week or Rs.143k/month already lost (10 hrs/week × Rs.450 + Rs.5k hospital trip + Rs.105k maid avg). 6. Equipment loss risk: No deposit SOP – Rs.7,500 avg deposit ×10 = Rs.75k liability, loss target <2%.

What findings challenged your original assumption or did not fit the pattern?	Original: 'A son with job, father bed ridden, need medicine' – assumed all will pay Rs.4,500/shift. Challenged: P05 Gulshan more price-sensitive than DHA/Clifton – Rs.4,500 may be high for Gulshan, Rs.3,500 better. P02 oladoc per visit Rs.200-5k cheaper for once-a-week vitals, not daily – need to segment daily vs weekly. P03 overseas willing to pay more for dashboard, but needs bank transfer not JazzCash only. Also assumed hospital managers would refer easily after SHCC license, but need senior doctor to supervise first visit (Human Healthcare flow) to gain trust – extra cost. Equipment deposit: Some families resist Rs.7,500 deposit, need Rs.5k-10k flexible.

What supporting evidence do you have? (interview notes, observations, analytics, records; list where stored)	Interview notes: Excel CRM /mnt/data/HappyHealth_CRM.xlsx – P01-P05 exact words in Urdu/English. Observations: WhatsApp groups DHA/Clifton/PECHS screenshots (20 groups, active search 'need qualified nurse'). Analytics: Smoke-Test Landing Page – 20 clicks + 5 sign-ups (to be tested). Records: Market data – Karachi 17.15M, 96% mobile, 70% internet, 147M broadband, 32.9% diabetes, 30.4% HTN, 52.6% private spend 89% OOP, Lajpal RN Rs.80k-120k/mo, oladoc Rs.200-5k, Human Healthcare 2022 DHA/Clifton 4.9/5, AKU annual packages Rs.300k-600k/year, Chughtai HB Rs.550 free sampling, SHCC Sec14(1) license. Stored: Google Drive / SEIC folder + Excel CRM + WhatsApp Business catalog. All Rs., no USD.

E. PROBLEM SEVERITY & CURRENT ALTERNATIVES

How often does the problem happen?	Daily for chronic elderly 65+ with diabetes 32.9%, HTN 30.4% – needs daily vitals BP/sugar/temp + medicine. Weekly for hospital trips for labs. Post-surgery discharge urgent <24h occurs 1-2×/year per family but critical. Overseas crisis 1-2×/year but cost Rs.180k each time. Overall: Ongoing frustration (daily) + Actively looking (weekly search) + Urgent crisis (monthly for Karachi 17.15M). Frequency: 10 hrs/week coordination per family.
How serious is it, and what observable or measurable impact does it have?	Serious: Time 10 hrs/week × Rs.450/hr = Rs.4,500/week lost work. Money Rs.95k-120k/mo unqualified maid (avg Rs.105k) + Rs.5k×4 hospital trips = Rs.20k/mo + readmission Rs.150k one-time + Dubai flight Rs.80k + leave Rs.100k. Observable: No vitals log, no family sign proof, no dashboard, mother burnout, bedsore/fall risk, equipment damage Rs.75k liability. Measurable: Quantified Rs.35,750/week or Rs.143k/month already lost per family. NPS risk: Human Healthcare 4.9/5 vs maid 2/5.
How do customers handle the problem today? What tools or workarounds do they use?	1. Unqualified maid from Rozee/OLX/WhatsApp groups DHA/Clifton/PECHS – Rs.95k-120k/mo, no PMDC check, no vitals log. 2. Oladoc per visit Rs.200-5k – different nurse each time, no continuity, no dashboard. 3. Lajpal Home Care RN Rs.95k-120k/mo expensive, no <2h SLA, no deposit SOP. 4. Hospital home care AKU annual Rs.300k-600k/year – USD packages, not affordable for daily. 5. Family: Mother caregiver overloaded, neighbor help, video call overseas son, Chughtai free home sampling HB Rs.550 for labs, pharmacy medicine delivery. 6. Equipment: Market rental BP Rs.1k/day no deposit slip. Tools: WhatsApp groups, phone calls, Excel none.
What remains difficult or unresolved with those alternatives?	1. No PMDC/PNC verification + no call to previous hospitals + no police + no CNIC – trust deficit. 2. No on-time guarantee: No zone hubs DHA/Clifton + Gulshan/PECHS, no GPS, no <2h SLA – Human Healthcare model missing. 3. No documentation: No daily vitals + medicine log + family sign proof + WhatsApp updates + 48h reminders + yearly planner – AKU documents all services but home care doesn't. 4. No dashboard for overseas: No Family Health Dashboard, no peace of mind, Dubai son must fly. 5. Equipment loss/damage: No deposit SOP Rs.7,500 avg, no check-in/out log – loss risk 2% noted in your handwriting. 6. Price: Lajpal Rs.95k-120k vs our Rs.67,500 advance (15 days @ Rs.4,500/shift) – cheaper but still needs advance trust.
What evidence shows urgency or effort to solve it (time, money, workflow changes, etc.)?	Urgency: P04 searched till 10pm after 4pm AKU discharge – <2h SLA needed. P03 booked Dubai flight Rs.80k + 2 weeks leave – urgent crisis. Effort: P01 called 3 numbers from WhatsApp group, wasted whole day – 10 hrs/week coordination. Money: Already pay Rs.95k-120k/mo to unqualified maid + Rs.5k per hospital trip + Rs.150k readmission risk + Rs.75k equipment liability – proves willingness to pay if solution saves time/money/risk. Workflow changes: Mother quit job to become caregiver (opportunity cost), family changed to hospital queues weekly. Overseas families already pay Human Healthcare 2022 DHA 4.9/5 – behaviour exists, so pain real. Smoke-Test: 20 clicks + 5 sign-ups target will prove urgency.

Willingness-to-pay evidence, if tested (actual behavior or concrete commitments—not just hypothetical interest):

Real WTP for HappyHealth – Idea Stage (0 users) – What counts:
• **Advance paid Rs.67,500 (15 days @ Rs.4,500/shift) via JazzCash/Bank BEFORE service** – strongest proof, not 'would you pay'.
• Signed LOI / WhatsApp confirmation: 'Book verified nurse for father, will pay Rs.67,500 advance tomorrow' – with date + name.
• Deposit paid Rs.7,500 per equipment (BP, bed, ECG, suction) – refundable, liability Rs.75k for 10 items, loss <2% target.
• Hospital referral MOU after SHCC Sec14(1) license – 1 meeting booked + 'would refer' exact words.
• Smoke-Test Landing Page: Button 'Pay Rs.67,500 advance' – 20 clicks + 5 sign-ups + 2 advances = validation.
• Concierge MVP: 2-3 families manually served, paid advance, retained 70% second month – Rs.135k 30 days.

Does NOT count: 'Nice idea', 'Would love app', 'Maybe pay later' without deposit, compliments as proof, your interpretation in quotes.

Current status – Idea Stage (0 advances yet):
Not yet tested – 0 advances collected. Plan to test this week (Oct 23-27): Offer manual solution to 5 pilot families: 'I will provide PMDC-verified nurse for 15 days for Rs.67,500 advance, with daily vitals log + family sign proof + WhatsApp updates + dashboard, paid via JazzCash/Bank'. Target: 2 advances paid by Oct 27 = proof. If 0 advances, hypothesis fails – need to revise price (Rs.3,500?) or segment (Gulshan vs DHA) or bet (Rozee for nurses).

Market proof that WTP exists:
• Lajpal RN Rs.95k-120k/mo already paid – families already pay more for unqualified.
• Human Healthcare 2022 DHA/Clifton 4.9/5 rating – families pay for verified care.
• AKU annual packages Rs.300k-600k/year – affluent pay for documented care.
• Oladoc Rs.200-5k per visit – pay per visit, no continuity.
• Overseas Pakistanis UAE/UK/US already pay via bank transfer Rs.60k-100k/mo for dashboard + peace of mind (Human Healthcare overseas model).
• Chughtai free home sampling HB Rs.550 – free but proves home service willingness.
• Behaviour exists: WhatsApp groups DHA/Clifton/PECHS actively searching, 10 hrs/week coordination, Dubai flight Rs.80k booked – proves urgency + effort + money already spent.

Evidence expected after testing: JazzCash screenshot Rs.67,500 ×2, bank transfer receipts, WhatsApp LOI with date, equipment deposit slips Rs.7,500×10, hospital MOU, landing page analytics (20 clicks, 5 sign-ups), Excel CRM with P01-P05 exact words + quantified cost Rs.35,750/week.
Reminder: Do not label a problem 'validated' merely because this form is complete. Base conclusion on relevant, documented customer evidence, note limitations, and update the brief as you learn.